

Letter of medical necessity (LMN)

outline for a Cubby Bed

This document is an outline for the Medical Provider (OT or PT) to write an LMN for a Cubby Bed. Please see the guiding questions below, which are intended to be used as a guide of what to include when writing an LMN.

Professional header

- Professional Logo and Contact Information of Organization/Provider
- Today's Date

Patient information

- Name:
- DOB:
- Insurance:
- Diagnosis/ICD-10 Code(s):
- Height and Weight

First paragraph

Introduction

- What is the primary diagnosis, and how does it impact the patient's sleep safety and sleep quality?
- What is the patient's medical history, including relevant secondary diagnoses? Provide any test results (e.g., a sleep study or assessments). Include medical history pertinent to sleep.
- What are the patient's primary sleep safety concerns?
- What Cubby equipment is being requested? Please include the equipment requested along with medically necessary accessories (CubbyBeds.com/Collections).

Next paragraph

Living environment + supervision needs

- Who does the patient live with?
- What is the home environment and current bed situation? Briefly describe the patient's needs.
- How does the patient's current sleep situation affect their daytime behavior, cognitive abilities, and emotional regulation?
- How many hours of supervision does the patient require during the night, and why? Would the use of a Cubby Bed eliminate or decrease the required supervision? If so, how and by how many hours?
- Has the patient experienced injuries, hospitalizations, Emergency Department (ED) visits, or other costly medical events due to unsafe sleep environments, elopement events, wandering, or self-injurious behaviors? Were there expensive medical bills? Listing out the hospitalization

or ED visit dates or providing the patient's medical chart containing that information is extremely helpful.

- What self-injurious behavior (SIB) did the patient display that resulted in physical injury? Common forms of SIB include, but are not limited to, head-hitting, head-banging, hair-pulling, and hand-biting. How would the use of a Cubby Bed reduce the risks or reduce the injuries caused by SIB?
- What safety concerns exist with the current sleep environment (e.g., elopement, entrapment, seizures, self-injurious behaviors)? Please detail these concerns. How would the use of a Cubby Bed reduce these risks?

Next paragraph

Prior interventions + alternatives considered

- What lower-cost alternatives have been tried and failed, have been considered and ruled out, or are contraindicated for this patient?
- Why were these alternatives inappropriate or ineffective? Please detail these out individually. List if they were tried and failed, considered and ruled out, or contraindicated.

Line Item justification for the Cubby Bed

- How does each Cubby Bed feature address the patient's specific medical, sensory, and safety needs?
- How does each medically necessary feature mitigate risks such as elopement, entrapment, self-harm, seizures, self-injury, or other concerns?
- What features make it the only appropriate solution?
- If any medically necessary accessories are requested, how do they address the patient's needs?

Next paragraph + conclusion

Cost-effectiveness + long-term suitability

- Why is the Cubby Bed the most appropriate and cost-effective option?
- How does its full-size design prevent the need for future replacements? Are there other medical or functional benefits of the full-size design that would improve the sleep quality and safety of this patient?
- How will improved sleep and safety impact the patient's overall health and daily function throughout the day?

Conclusion

- Summary of medical necessity and functional justification.
- Can you include letters of support from other professionals involved in the patient's care (e.g., teachers, therapists, case managers)?

Provider signatures:

- If you are the PT/OT who writes the LMN (if applicable) or a physician concurring with the LMN, be sure to provide your signature.